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Name:	Mr.Abhishek Roy	**Age/Gender:**	43 Years/Male
Referred By:	N.A	**Client Name:**	N.A
Collection Date:	28-12-2022 07:56:00	**Report Release Date:**	28-12-2022 20:22:58

MediBuddy Full body Health Checkup

No.	Investigation	Observed Value	Unit	Biological Ref. Interval
	Complete Blood Count			
	Erythrocytes			
1	Total RBC	5.7	10^6/µL	4.1-6
2	Hemoglobin	14.7	g/dL	13 -17.5
3	Hematocrit (PCV)	48.4	%	33-57
4	Mean Corpuscular Volume (MCV)	84.9	fL	80-96
5	**Mean Corpuscular Hemoglobin (MCH)**	**25.7**	**pg**	**27.5-33.2**
6	**Mean Corpuscular Hemoglobin Concentration (MCHC)**	**30.3**	**g/dL**	**30.4-34.5**
7	**Red Cell Distribution Width (RDW-CV)**	**15.7**	**%**	**12-15**
8	Red Cell Distribution Width-SD(RDW-SD)	45.5	fl	30-64.5
9	Nucleated Red Blood Cells	0.01	cells/µL	0 - 1.36
10	Nucleated Red Blood Cells Percentage	0.1	%	0-4
	Platelets			
11	Platelet Count	250.0	10^3/µL	150-450
12	Mean Platelet Volume (MPV)	9.5	fL	6 - 12
13	Platelet Distribution Width (PDW)	17.2	%	15.5-18.3
14	Plateletcrit (PCT)	0.238	%	0.12-0.37

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Remarks

WBC-Leucocytosis

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No.	Investigation	Observed Value	Unit	Biological Ref. Interval
	Urine Complete			
	Physical Examination			
1	Specific Gravity	1.015		1.000 - 1.035
2	pH-value	6.0		5 - 8
3	Color	Pale Yellow		
4	Clarity	Slightly Hazy		
	Bio-Chemical Examination			
5	Leukocytes	Negative		Negative
6	Nitrite	Negative		Negative
7	Protein	Negative		Negative
8	Urine Glucose	Negative		Negative
9	Ketones	Negative		Negative
10	Urobilinogen	Negative		Normal
11	Bilirubin	Negative		Negative
12	Blood	Negative		Negative
	Microscopic Examination			
13	Erythrocytes	0-1	Cells/hpf	< 4

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Liver Function Test - LFT

No.	Investigation	Observed Value	Unit	Biological Reference Interval
1	Bilirubin Total Serum, Method: Jendrassik Grof	1.0	mg/ dL	0.2-1.2
2	Bilirubin Direct Serum, Method: Diazotization	0.05	mg/ dL	0.01 - 0.4
3	Bilirubin Indirect Serum, Method: Calculated	0.95	mg/dL	0.01-1.0
4	**Aspartate Transaminase (AST/SGOT)** Serum, Method: UV Kinetic with P5P	**74.3**	**U/ L**	**<50**
5	**Alanine Transaminase (ALT/SGPT)** Serum, Method: UV Kinetic with P5P	**53.0**	**U/ L**	**<50**
6	**Alkaline Phosphatase** Serum, Method: AMP – pNPP Kinetic	**146.0**	**U/L**	**30 - 130**
7	Total Protein Serum, Method: Biuret end point	7.29	g/dL	6.4 - 8.2
8	Albumin Serum, Method: Bromocresol Purple (BCP)	4.01	g/dL	3.4 - 5
9	Globulin Serum, Method: Calculated	3.28	g/dL	1.9-3.9
10	A/G ratio	1.22		1.0 - 2.0

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No.	Investigation	Observed Value	Unit	Biological Reference Interval
1	Calcium Serum, Method: O cresolphthalein complexone	9.5	mg/dL	8.5 - 10.5

Interpretation

Calcium is the most abundant and one of the most important minerals in the body. It is essential for cell signaling and the proper functioning of muscles, nerves, and the heart. Calcium is needed for blood clotting and is crucial for the formation, density, and maintenance of bones. The causes of hypercalcemia include Hyperparathyroidism and dietary intake. Low blood protein levels, especially a low level of albumin, which can result from liver disease or malnutrition, both of which may result from alcoholism or other illnesses.

Serum Electrolyte Profile

2	Sodium Serum, Method: Indirect ISE	136.28	mmol/L	136 - 145
3	**Potassium** Serum, Method: Indirect ISE	**5.22**	**mmol/L**	**3.5 - 5.1**
4	**Chloride** Serum, Method: Indirect ISE	**94.7**	**mmol/L**	**98 - 107**

Interpretation

The electrolyte panel is used to identify an electrolyte, fluid, or pH imbalance (acidosis or alkalosis). It is frequently ordered as part of a routine physical. Electrolyte measurements may be used to help investigate conditions that cause electrolyte imbalances

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No.	Investigation	Observed Value	Unit	Biological Reference Interval
1	**Phosphorous** Serum, Method: Phosphomolybdate method	**5.1**	**mg/ dL**	**2.5 - 4.9**

Remarks

Kindly correlate clinically ,Rule out the pre-analytical variables.

Interpretation

Phosphorus comes into the body through the diet. About 70-80% of the body's phosphates combine with calcium to help form bones and teeth, another 10% are found in muscle, and about 1% is in nerve tissue. Low levels of phosphorus (hypophosphatemia) in the blood may be due to or associated with Hypercalcemia, especially due to hyperparathyroidism ,Overuse of diuretics, Malnutrition, Alcoholism, Severe burns, Diabetic ketoacidosis (after treatment), Hypothyroidism, Hypokalemia,Chronic antacid use, Rickets and osteomalacia (due to vitamin D deficiencies). Higher levels of phosphorus (hyperphosphatemia) in the blood may be due to or associated with Kidney failure, Liver disease, Hypoparathyroidism, Diabetic ketoacidosis (when first seen), increased dietary intake (phosphate supplementation)

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No.	Investigation	Observed Value	Unit	Biological Reference Interval
	Lipid Profile			
1	Total Cholesterol Serum, Method: Cholesterol oxidase,esterase,peroxidase	210.9	mg/dL	Desirable: <200; Borderline high = 200-239; High: > 240
2	**Triglycerides** Serum, Method: Enzymatic, end point GPO-POD	**205.1**	**mg/dL**	**Desirable: <150** **Borderline High: 150 - 199** **High: > 200 - 499**
3	HDL-Cholesterol Serum, Method: Enzymatic Immunoinhibition	55.7	mg/dL	30 - 60
4	LDL- Cholesterol Serum, Method: Enzymatic selective Protection	117.8	mg/dL	Optimal: <100; Near Optimal: 100-129; Borderline High: 130-159; High: 160-189; Very high: >190
5	Cholesterol/HDL ratio Serum, Method: Calculated	3.79		Optimal: <3.5 Near Optimal: 3.5 - 5.0 High >5.0
6	**VLDL Cholesterol** Serum, Method: Calculated	**41.02**	**mg/dL**	**6 - 40**
7	LDL /HDL ratio Serum, Method: Calculated	2.11		Optimal: <2.5 Near Optimal: 2.5-3.5 High >3.5
8	Non HDL Cholesterol Serum, Method: Calculated	155.20	mg/dl	Desirable: <130 Borderline high: 130-159 High : 160-189 Very High :>190

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No.	Investigation	Observed Value	Unit	Biological Reference Interval
	Thyroid Profile - Total T3,Total T4,TSH (TFT)			
1	Total T3 Serum, Method: CLIA	101.55	ng/dL	60 - 200
2	Total T4 Serum, Method: CLIA	8.69	µg/dL	4.5 - 14.5
3	TSH (Thyroid Stimulating Hormone) Serum, Method: CLIA	4.955	µIU/ml	0.35 - 5.5

Interpretation

1. Triiodothyronine (T3) is produced by the thyroid gland and along with thyroxine (T4) help control the rate at which the body uses energy. Elevated T3 denote hyperthyroidism while low levels indicate hypothyroidism.

2.The most common causes of thyroid dysfunction are related to autoimmune disorders. Graves disease causes hyperthyroidism, but it can also be caused by thyroiditis, thyroid cancer, and excessive production of TSH. Total T3 is used to assess thyroid function.

3. Elevated T4 levels may indicate hyperthyroidism. They may also indicate other thyroid problems, such as thyroiditis or toxic multinodular goiter. Abnormally low levels of T4 may indicate: dietary issues, such as fasting, malnutrition, or an iodine deficiency, medications that affect protein levels, hypothyroidism, illness.

4. Thyroid-stimulating hormone (TSH) stimulates the production and release of T4 (primarily) and T3. They help control the rate at which the body uses energy and are regulated by a feedback system. Most of the T4 circulates in the blood bound to protein, while a small percentage is free (not bound).

5. Lab has estimated Total T4 reference intervals that are specific for India, using the indirect sampling technique following

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No.	Investigation	Observed Value	Unit	Biological Reference Interval
	Kidney Function Test			
1	Uric Acid Serum, Method: Uricase, UV	6.5	mg/ dL	2.1 - 7.5
2	BUN (Blood Urea Nitrogen) Serum, Method: Calculated	7.34	mg/dL	3.3 - 18.7
3	Urea Serum, Method: Urease-GLDH	15.7	mg/dL	7 - 40
4	Creatinine Serum, Method: Alkaline picrate kinetic	0.6	mg/dL	0.5 - 1.3
5	BUN/Creatinine ratio Serum, Method: Calculated	12.23		4.0 - 21.5
6	Urea/Creatinine Ratio Serum, Method: Calculated	26.17		10 - 50

Interpretation

A renal function panel could be ordered when a patient has risk factors for kidney dysfunction such as high blood pressure (hypertension), diabetes, cardiovascular disease, obesity, elevated cholesterol, or a family history of kidney disease. A renal function panel may also be ordered when someone has signs and symptoms of kidney disease, though early kidney disease often does not cause any noticeable symptoms. It may be initially detected through routine blood or urine testing. Renal function panel results are not diagnostic but rather indicate that there may be a problem with the kidneys and that further testing is required to make a diagnosis and determine the cause. Results of the panel are usually considered together, rather than separately

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No.	Investigation	Observed Value	Unit	Biological Reference Interval
1	Glucose (Fasting) Fluoride Plasma(F), Method: Hexokinase	98.1	mg/dL	Normal : >70 - 100 Pre - Diabetes: 101 - 126 Diabetes: > 126

Interpretation

Glucose is the primary energy source for the body's cells and the only energy source for the brain and nervous system.High levels of glucose most frequently indicate diabetes, but many other diseases and conditions can also cause elevated blood glucose. Hypoglycemia is characterized by a drop in blood glucose to a level where first it causes nervous system symptoms (sweating, palpitations, hunger, trembling, and anxiety), then begins to affect the brain (causing confusion, hallucinations, blurred vision, and sometimes even coma and death).

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QUALITY POLICY

GENERAL DIAGNOSTICS INTERNATIONAL (P) Ltd. maintains the highest standards of quality control in all aspects of laboratory work. The purpose of our laboratory's Quality Management System is to ensure that:

- Principles of all accreditations, including that of NABL - ISO1518:2012 (National Accreditation Board of Laboratories) are adhered for each test in the scope of the accreditation, and beyond.
- Test methods, processes and control mechanisms are timely updated and fully validated to ensure the accuracy and reliability of our test results.

The objectives of our Quality Control system are:

- Use Bar-Coded operations to enable full traceability throughout the sample flow process and to ensure sample handling procedures and environmental conditions are managed well and there is no or minimal affect on the results.
- Continually improve the practices of our clients, franchise partners, associate doctors, clinics and hospitals and monitor their training needs. Be proactive in identifying gaps in the processes being followed. Guide them to ensure that the patients are served in the best possible way.
- Report the results with accuracy and clarity in a timely manner. Do a root cause analysis whenever there is a deviation against protocols and find solutions to the identified causes.
- Ensure a continual enhancement, implementation and maintenance of the quality system and seek improvement in the effectiveness of the quality system from experts at regular intervals.
- Meet and exceed expectations with respect to turn-around time, sample collection hygiene & reliability of service.
- Ensure that each test is performed by qualified and trained staff. Provide opportunities to the staff so that they can increase their knowledge and use the same for self and organizational betterment.
- Ensure that the equipment used are best in class, properly maintained and calibrated and where possible, measurements are traceable to recognized standards. Also explore methods which may lead to improvement in equipment performance and methodologies used for conducting tests.
- Enable technology upgrades to achieve higher accuracy and reduced complexities.
- Use internal audits and other checks to ensure the quality system complies with requirements; ensure problems are investigated promptly, root cause(s) established and effective action taken to prevent a recurrence.
- Have a smooth communication mechanism to ensure information is made available as rapidly as possible to those who need it, both internal and external to the organization.
- Monitor, help and support our franchise and service partners to be sensitive on all aspects of service delivery and to ensure quality standards are followed with no exceptions.

CONDITIONS of REPORTING

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